ASTER PRIME HOSPITAL, PUNE

Department of General Surgery — Discharge Summary

Patient: Ramesh Kulkarni | Age: 58 M | UHID: APH-2026-10482
Admission: 12-Apr-2026 | Discharge: 15-Apr-2026
Treating Surgeon: Dr. A. Deshmukh (Reg. MH-48219)

DIAGNOSIS

Post-laparoscopic sepsis with delayed recognition; acute kidney injury stage 2; community-acquired pneumonia on discharge radiograph.

HISTORY OF PRESENT ILLNESS

Patient underwent diagnostic laparoscopy for chronic abdominal pain on 12-Apr-2026. From post-operative day 1 he reported persistent fever up to 39.2 C, tachycardia of 118/min and worsening abdominal tenderness. C-reactive protein rose from 42 mg/L to 214 mg/L over 48 hours, and procalcitonin was reported at 18.4 ng/mL on post-operative day 2. Despite these markers the patient was advised discharge on 14-Apr-2026 without repeat imaging or surgical re-exploration. He was readmitted on 15-Apr-2026 with hypotension, oliguria and confusion, at which point septic shock was recognised.

COURSE IN HOSPITAL (READMISSION)

Fluid resuscitation, broad-spectrum antibiotics and vasopressor support were started on admission. CT abdomen showed a contained peritoneal collection which was drained percutaneously. Creatinine peaked at 3.1 mg/dL before settling to 1.4 mg/dL. ICU stay was 4 days. The patient was discharged on 22-Apr-2026 with advice for weekly renal follow-up and a six-week antibiotic course.

FOLLOW-UP ADVICE

1. Nephrology review after 2 weeks with serum creatinine and electrolytes.
2. Surgical wound review on 29-Apr-2026.
3. Readmit immediately if fever exceeds 38 C or urine output falls.

SUMMARY FOR THE TREATING TEAM

The interval between the rising inflammatory markers on post-operative day 1-2 and the decision to discharge constitutes the key clinical decision under review. No repeat imaging was documented before the first discharge.

Digitally signed — Dr. A. Deshmukh, 24-Apr-2026

This document is a synthetic sample prepared for demonstration purposes.